

Knowledge Base, Final Session

Closing the last open items, and reviewing your digital presence.

With: Dr Gina Schoeman and Jess Morrell (WILBA)

Aim: Confirm what we have already captured, close the final knowledge base items, lock the clinical safety and escalation detail, and review your digital presence.

FIRST, A QUICK RECAP

Confirmed so far

Everything below is already captured from our earlier sessions and your notes. Please flag anything that has changed, otherwise it is locked and we move on.

CONSULTATIONS AND BLOODS

Consultations: concise 45 min 350, standard 60 min 500, comprehensive 90 min 750. Follow-ups: 30 min 250, 15 min brief 125, 45 min 350.

Bloods: mandatory before the first consultation, via TDL (Wimpole Street), home kit, or own results. Menstruating patients test days 18 to 22. Consultation about one month after the test.

Blood panels and prices: received (GS1 to GS13). A couple of quick confirms below.

APPOINTMENTS, RECALL AND LETTERS

Cancellations: 2 working days notice or the full fee applies.

Recall: reminder for a planned follow-up 6 weeks ahead. Re-engagement flow for overdue patients, who are also given to the admin team as a list and an email.

Appointment reminders: handled by Semble, so we do not duplicate them.

Post-consultation letter: ready within 5 working days, with agreed holding responses.

CLINICAL SAFETY, SO FAR

Results: always go to the clinician, never sent to the patient, unless the patient supplies their own.

Routing: all clinical questions reviewed by a clinician; appointment date changes always handled by a PA.

TONE, COVERAGE AND OUT OF HOURS

Tone: warm, patient centred and polished; no negative wording; no emojis; no comparisons to other clinics; sign off "Best wishes, The Schoeman Clinic Team".

Coverage: shared PA inbox, office hours Monday to Friday 9 to 5. Agreed out-of-hours and escalation holding messages in place.

The escalation boundary

The framework is agreed: operational stays with the team, anything clinical goes to a clinician, urgent gets an immediate human. These are the last details to make it watertight, in your words.

1. **Medication questions.** The exact holding line a patient receives for "should I change my dose", "should I stop my medication", "is this side effect normal", while a clinician takes over.
2. **Safeguarding and crisis.** A mental health crisis, self harm, or an emergency. What is the patient told immediately (999 / 111), which words or phrases must always trigger an urgent flag, and who is notified, and how.
3. **Sounds admin, is clinical.** Two or three real examples ("can I take X with Y", "is this safe in pregnancy") so the line is unmistakable.
4. **Never answer.** The short list of anything the assistant must never answer, under any circumstances.
5. **Repeat prescriptions.** Exactly what the team checks before approving a repeat (the treatment plan), and what to do if the patient is overdue for a follow-up.

Doctors, new and oversight

1. **New doctors.** Dr Nathan, Dr Sonia and Dr Nikita are joining. What should the assistant say about them when a patient asks?
2. **Oversight as you grow.** So clinical questions reach the right treating doctor with the oversight you want, how would you like to stay aware of open queries: a short daily summary per doctor, a shared view you can check at any time, or an alert only when something is unanswered beyond a set time?

The last operational details

Rapid answers are perfect. Rough bullets and "roughly how I would say it" are exactly right.

SERVICES AND REFERRALS

- Top 3 to 5 reasons patients enquire (your data shows hormone, gut, then skin), and which are more niche?
- Anything patients ask for that you do not offer, and where you refer them instead?
- IV therapy and injectables: by appointment or walk in, and at which location?
- One off screenings (cardiac, cancer, biological age): standalone, or only after a consultation?

BLOODS AND PAYMENTS

- Confirm the fasting requirement on the panels (currently a draft), and the few add-on prices still marked to be confirmed.
- Rough turnaround time for results, and the refund position if a patient pays for bloods but does not go ahead.
- A simple line for "why the roughly one month wait after bloods", and a reassuring line for "is this payment link genuine".

PRESCRIPTIONS AND PHARMACIES

- When Zen is used versus a compounding pharmacy: always the clinician's decision?
- Preferred wording when a patient says they have not received their pharmacy link.
- Happy for the assistant to share the Gut Map code and a "please check your spam folder" line for Nordic, Vibrant and Natural Dispensary?

KEEPING IT CURRENT

- How often do prices or partners change, and the easiest way for you to send us updates?
- Once live, the simplest way for the team to flag anything the assistant answers incorrectly?

PART 4

Digital presence and social

WHAT WE REVIEWED

A look across your current presence: Instagram, Facebook, the website, and your Google presence, with a few clear, low effort opportunities to lift it. (Walk through the audit [here](#).)

YOUR GOALS AND VISION

- What would success look like for your online presence over the next 6 to 12 months?
- Who is the ideal patient you would most like to attract?
- How do you feel about appearing on camera, and how much time could you realistically give to content?
- How would you like us to work alongside Nadia, so efforts are joined up rather than duplicated?
- Do you have brand assets we should work from: logo, fonts, photography, a tone or brand guide?

ACCESS WE WOULD VALUE (VIEW OR ANALYTICS LEVEL)

- Meta Business Manager (Instagram and Facebook), also needed for the WhatsApp Business verification.
- Website analytics (Google Analytics) and the Google Business Profile.
- Any current social scheduling tool, and the brand assets above.

Good news to share: the patient consent and privacy notice is complete and with the data team for sign off, and the "no em dashes" preference is built into all patient-facing copy.

Prepared by WILBA for The Schoeman Clinic. Agreed actions and any changes to capture live on the call.